



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : صفاء احمد البدوي عبدالجابر

العنوان : الخرطه الجديده

الحي / المدينة : الجيزه

المحافظة : الجيزه

تاريخ تسجيل الإستماره : 07-10-2026

رقم البطاقة : 27211192100103 رقم التليفون : 01122140613

العمر (تاريخ الميلاد) : 19-11-1972 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي : _____

المقرر بما فيه : _____

التوقيع : (-----) : _____

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : _____

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني

الاسم : د/ندى حسن

الاسم : د/نرمين رمضان

الوظيفة : _____

الوظيفة : _____

الوظيفة : _____

التوقيع : _____

التوقيع : _____

التوقيع : _____

تعتمد صحة البيانات المرفقة

رئيس اللجنة ومدير

المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس

وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / _____ د. / _____ د. / _____ د. / _____

قرار اللجنة

التشخيص النهائي : _____

خطة العلاج : _____

المستشفى : _____

قسم : _____

جهة العلاج : _____

نفقات : _____

ملاحظات : _____

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: صفاء احمد البدوي عبدالحام National ID: 27211192100103	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



MDT FORM

14 / 8 / 2026 G

Name	هفاء أحمد السيد	MRN	1900/C-60
Age	52 yrs	ID	0114320578
Menopausal status	Post menopause	Phone	0111191111
NRS SCORE		Marital status	D
BMI		Family history	
CO morbidity	M → Fre. S → Fre.	REASON OF MDT	New Case

Clinical Data

BX: Right breast: IDC G III.

ER: 0/8 PR: 2/8 HER 2: -ve Ki67: 60%

Sonomammography: ACR 4.

→ Right breast mass 13.9 x 2.6 cm

Mets Workup:
at 11-12 o'clock, associated with
large area of heterogenous parenchyma

→ Few Suspicious. at level 2

R → large mass occupying
most of the breast.



TNM
T_{4b} N₁ M₀

Recommendations:

CT(AB) → Signal 2 LFL. for Triphasic CT.

then for NAT by MRM (impl) Requested.

MDT OUTCOME & PLAN

for Triphasic C.T on liver

Start ACX 4 till result of Triphasic
take photo

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name: صفاء احمد البدوي MR. : 1023	
Detailed Medical Report	
السيد	
تحية طيبة وبعد,,	
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها	
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :
وبعد الكشف والفحوصات والمتابعة وجد انها The patient is a 53-year-old with a history of right-sided breast cancer, clinical stage T4 N1 M0, grade 3, triple-negative phenotype, status post-mastectomy (REC 4). The current adjuvant regimen consists of weekly Taxol 130 mg and Carboplatin dosed to AUC 5, initiated in week 6 of therapy. On examination, a right upper outer quadrant mass measuring approximately 4 × 5 cm was noted. The patient presented for administration of week 7 Taxol 130 mg and Carboplatin 200 mg (AUC 2). A single dose of G-CSF was supplied for the upcoming week 8 and week 9 complete blood count monitoring, and the pharmacy processed the order accordingly. Pharmacy documentation confirmed that week 8 treatment was scheduled with Taxol plus Carboplatin at AUC 2. The daycare pharmacy recorded that week 9 treatment would consist of Taxol 130 mg and Carboplatin at AUC 2, to be administered at the next clinic visit. Clinical pharmacy services noted a delay in the week 10 visit due to Ministry of Health approval issues. The plan was to give Taxol 130 mg and Carboplatin (AUC 2) 220 mg at the upcoming clinic appointment, with new laboratory studies to be obtained. During the clinical pharmacy encounter, the patient's total leukocyte count was $2 \times 10^9/L$ and neutrophil count $1.2 \times 10^9/L$; prophylactic Neupogen was prescribed. The current plan is to proceed with the scheduled week 10 Taxol and Carboplatin administration pending lab results and approval.	
تحريراً في ,	
التاريخ :	19-09-2026
الطبيب المعالج :	dina makram
يعتمد ,	

Patient Name: Safaa Ahmed El-Badawy Abd Al Gaber Abu Taleb	Date : 27/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 52 Year(s)
Medical ID: 1900/2025	Pat. ID: 100065

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- Right breast rather well-defined enhancing lesion measuring 4.5 x 3.3 cm is noted.
- Multiple right axillary pathologically enlarged lymph nodes are noted largest measuring 3.7 x 2.7 cm.
- No pulmonary nodules, masses or cavitory lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or pathological lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing few left hepatic lobe hypodense non enhancing focal lesions largest measuring 1 x 1 cm at segment 2.
- No dilated biliary radicles.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- No pelvic masses.
- No abdominal or pelvic pathological lymphadenopathy.
- No ascites.

Opinion:

- ❖ Right breast lesion as described, for sonomammographic & histopathological correlation.
- ❖ Multiple right axillary pathological lymph nodes for histopathological correlation.
- ❖ Left hepatic lobe focal lesions for Triphasic CT/Dynamic MRI

Patient Name: Safaa Ahmed El-Badawy Abd Al Gaber Abu Taleb	Date : 27/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 52 Year(s)
Medical ID: 1900/2025	Pat. ID: 100065

Dr. Zeyad Badawy,MSc,FEBR.

Radiology consultant.

Not Approved

Patient Name: Safaa Ahmed El-Badawy Abd Al	Date : 14/06/2025
Abu Taleb	
Exam : MAMMOGRAPHY BOTH	Age : 52 Year(s)
Medical ID :	Pat. ID: 100065

BILATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

- Breast density: Moderate dense breast tissue...Matching ACR C.
- Thickened right breast skin associated with focal asymmetry in the right upper outer quadrant and irregular shaped mass.
- No detected suspicious mass density, architectural distortion or pathological micro-calcification in the left breast.
- Normal skin and nipple on the left side.

Complementary breast ultrasound:

- Homogenous (predominantly glandular) echo-texture of both breasts.
- Corresponding to the mammographic findings: An irregular shaped non-circumscribed hyperechoic mass is seen in the right breast at 11-12 o'clock position, zone 1, it measures (2.5 x 1.6 cm) associated with large area of dense heterogeneously enhancing hyperechoic extending in between 10-12 oclock position, zone 1.
- Thickened right breast skin (6 mm).
- Normal left skin and nipple.
- No detected suspicious solid or cystic masses in the left breast.
- few (at least 3 in number) suspicious looking level 2 right axillary LN measuring 2.6 x 1.8 cm.
- No detected significant left axillary LNs.

IMPRESSION:

- Right breast: UOQ highly suspicious multifocal malignant process, considering the extreme dense breast contrast study is recommended to delineate the Actual extent BIRADS 5 category.
- Left breast: normal BIRADS 1 Category.

RECOMMENDATION:

Dr/MONA ZAKY, MD.

Chief of diagnostic radiology-breast imaging unit -Mansoura university

Signature Dr. Mona Zaki

Patient Name : الأستاذ / صفاء احمد البدرى	Request No. : 13001475
Patient No. : 27211192100103	Request Date : 17/06/2025
Path No. : 2051/25	Verification Date : 25/06/2025
Sex : Female	Financial Category : اورام دار السلام - مبادره
Age : 52 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data
Right breast mass

Nature of specimen
Tru cut biopsy, collected at 17/6/2025, 10:30 am

Gross examination

Multiple grayish white tissue cores up to 1 cm, totally submitted.

IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (3+3+2) Grade III
- * DCIS: No
- * Angioinvasion: Couldn't be detected in the received biopsy
- * Perineural invasion: No
- * IPT (Positive control showed appropriate reactivity) :
- * ER: No nuclear reaction in tumor cells (Allred score 0/8)
- * PR: Weak nuclear reaction in <1 % tumor cells (Allred score 2/8)
- * HER2: Moderate complete membrane reaction in>10 % of tumor cells (score 2+)
- * SISH :Average Her2 copy No .3.7 , HER2/CEP17 ratio :1.2
- * Ki 67: 60 % nuclear reaction of tumor cells

Diagnosis

Verified by :
Dr.Heba Hasan

Printing Date : 05/07/2025 10:19:38

Page 1 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / صفاء احمد البدرى
Patient No. : 27211192100103
Path No. : 2051/25
Sex : Female
Age : 52 Y
Referreing Doc :

Request No. : 13001475
Request Date : 17/06/2025
Verification Date : 25/06/2025
Financial Category : اورام دار السلام - ميانرو
Department : Out Patient
Registered by : .marwa mohamed youss

Pathology Report

Right breast mass, Tru cut biopsy:
Invasive duct carcinoma, Grade III
IPT:

- * ER: Negative (score 0/8).
- * PR: Negative (score 2/8).
- * HER2: Equivocal (score 2+).
- * SISH : Not amplified
- * Ki67: Positive in 60% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghoneim
Dr. Heba Hassan

Verified by :
Dr. Heba Hasan

Printing Date : 05/07/2025 10:19:38

Page 2 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Safaa Ahmed Elbadwy	Patient ID: 27211192100103
Report Date: 20/10/2025 05:22:51 PM	Age: 53 Years, 9 Months, 20 Days

MRI EXAMINATION OF THE ABDOMEN AND PELVIS

Technique:

- Axial T1 (in and opposed phases), T1 fat-sat, T2 & DWI
- Coronal and sagittal T2 WI
- Dynamic post-contrast series

Findings:

A reported case of breast cancer;

- Average sized liver showing smooth outlines and homogeneous parenchymal signal and enhancement. Few bilobar hepatic cysts are seen displaying low T1 and high T2 signals with no diffusion restriction or post-contrast enhancement – largest is seen in segment IVa/II measuring 12 mm.
- Patent homogeneously opacified portal vein and its main branches.
- Prominent CBD and central intrahepatic bile ducts are noted.
- The gall bladder show no calcular filling defects. Focal fundal adenomyomatosis is suggested.
- Normal MRI appearance of the spleen, pancreas, adrenal glands, kidneys (apart from bilateral small cortical cysts), aorta and IVC.
- The urinary bladder shows no gross masses, calcular filling defects or diverticulae.
- Bulky uterus with diffusely thickened junctional zone in keeping with diffuse adenomyosis. A small fundal intra-mural fibroid measuring 16 mm is seen. Small cervical Nabothian cyst is noted.
- No adnexal masses or sizable cystic lesions.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- No ascites.
- Small midline anterior abdominal wall hernia containing fat is seen supra-umbilical in location.

OPINION:

- Few bilobar hepatic cysts as above detailed
- No metastatic abdomino-pelvic lesions appreciated

Dr. Dina Osama
Dr. Basma Mamdouh

Dr. Eman Zaki, MD

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